

Redacted



Name: [REDACTED] Age/Sex: /F Location: [REDACTED]
Acct#: [REDACTED] Unit#: [REDACTED] Status: Room/Bed:
Reg: Disch: M.D.

Specimen: Received: Status: Req#: [REDACTED]
Collected: Sp type: SURGICAL P
Subm Dr: 1.D.

PREOPERATIVE DIAGNOSIS

CANCER OF THE RIGHT BREAST AND AXILLARY NODES

OPERATION PERFORMED

DATE:
DOCTOR(S):
PROCEDURE: MASTECTOMY MODIFIED RADICAL

TISSUE REMOVED

- A. RIGHT BREAST
- B. RIGHT AXILLARY CONTENTS

GROSS DESCRIPTION

RECEIVED IN 2 PARTS.

PART A RECEIVED LABELED [REDACTED] RIGHT BREAST DOUBLE BLACK STITCH 12 O'CLOCK AT NIPPLE IS A 1,416 GRAM MASTECTOMY MEASURING 28.5 CM FROM MEDIAL TO LATERAL, 26.5 CM FROM SUPERIOR TO INFERIOR, AND UP TO 6 CM FROM ANTERIOR TO POSTERIOR. A SUTURE DENOTES 12 O'CLOCK. THE NIPPLE IS FLAT WITH THE SURROUNDING AREOLA MEASURING 6 CM IN DIAMETER. IN THE UPPER OUTER QUADRANT 6 CM FROM THE NIPPLE THERE IS A 0.3 CM ULCERATION. AT THE MOST LATERAL ELLIPTICAL TIP OF THE SKIN IS A 2ND SMALL ULCERATION 0.5 CM IN GREATEST DIMENSION. THIS IS 16 CM FROM THE NIPPLE. THE SKIN ELLIPSE MEASURES 25.6 X 11.5 CM. THERE IS A PALPABLE MASS AT 12 O'CLOCK. "MRM" IS WRITTEN ON THE SPECIMEN AT 12 O'CLOCK. THE ANTERIOR MARGIN IS MARKED BLUE AND THE DEEP IS MARKED BLACK. SECTIONING REVEALS THE MAJORITY OF THE BREAST TO CONSIST OF BLAND YELLOW FATTY TISSUE WITH FINE FIBROUS BANDS. THERE ARE BROADER AREAS OF FIBROSIS IN THE MORE CENTRAL ASPECT OF THE SPECIMEN. IN THE 12-1 O'CLOCK AREA THERE IS A FIRM GRAY-TAN MASS WHICH IS GREATER THAN 1 CM FROM THE DEEP MARGIN MEASURING 5.8 CM FROM SUPERIOR TO INFERIOR, 2.5 CM FROM ANTERIOR TO POSTERIOR, AND 4 CM FROM MEDIAL TO LATERAL. CENTRALLY THERE IS AN IRREGULAR AREA OF RED-BROWN DISCOLORATION. THIS IS ALSO GREATER THAN 1 CM FROM THE OVERLYING SKIN. A COIL CLIP IS IDENTIFIED WITHIN THE LESION. SECTIONS ARE SUBMITTED AS FOLLOWS: A1--NIPPLE, A2--SKIN ULCERATIONS AND 12 O'CLOCK SKIN MARGIN, A3--DEEP MARGIN TO CENTRAL LESION, A4--ANTERIOR MARGIN TO CENTRAL LESION, A5 THROUGH A7--A SUPERIOR TO INFERIOR CROSS SECTION THROUGH THE LESION AT LEVEL OF CLIP, A8--THE MOST MEDIAL ASPECT OF LESION, A9--MOST LATERAL ASPECT OF LESION, A10 AND

1CD-0-3

carcinoma, infiltrating lobules mixed
with other types 8524/3
(Signet ring)

Site: breast, nos 250.9

hw
10/26/12

Patient: [REDACTED]

(Continued)

Specimen:

Received:

Status:

Req#: [REDACTED]

Collected:

Sp type: SURGICAL P

Subm Dr:

M.D.

GROSS DESCRIPTION

(Continued)

11--ADDITIONAL SECTIONS OF MASS, A12--UPPER INNER QUADRANT 4 CM FROM LESION, A13--UPPER OUTER QUADRANT 5 CM FROM LESION, A14--LOWER OUTER QUADRANT 5 CM FROM LESION, A15--LOWER INNER QUADRANT 4 CM FROM LESION, A16--TISSUE DEEP TO NIPPLE AT MEDIAL EDGE OF LESION.

NOTE: ~~TISSUE IS SUBMITTED PER CLINICAL BREAST CARE PROJECT RESEARCH~~ PROTOCOL OF THE TUMOR LABELED P1-P10 AND MIRROR IMAGE SECTIONS OF THE NIPPLE (A1) AND THE RANDOM QUADRANT SECTIONS (A12-A15) ARE SUBMITTED FROZEN IN OCT.

PART B RECEIVED LABELED [REDACTED] AXILLARY CONTENTS ARE 4 FRAGMENTS OF YELLOW-PINK FATTY TISSUE TOGETHER MEASURING 10 X 7.5 X 1.5 CM. THE SPECIMEN IS THEN EXAMINED FOR LYMPH NODES. ALONG ONE EDGE OF THIS SPECIMEN THERE ARE STRANDS OF RED MUSCLE FIBERS. EXAMINATION REVEALS 8 NODAL STRUCTURES. THESE ARE SUBMITTED AS FOLLOWS. FOUR (4) OF THESE MEASURE LESS THAN 0.5 CM AND ARE SUBMITTED AS B1. ONE OF THE LARGER NODES IS GROSSLY POSITIVE, ONE-HALF IS SUBMITTED IN B2, THE MIRROR IMAGE AS P11 PER THE CLINICAL BREAST CARE PROJECT RESEARCH PROTOCOL. THE REMAINING 3 LARGE NODES ARE EACH SUBMITTED BISECTED AND SEPARATELY LABELED B3 THROUGH B5. B6 IS REPRESENTATIVE MUSCLE. A BIOPSY CLIP IS NOT GROSSLY IDENTIFIED.

COMMENT: THIS CASE IS IN COMPLIANCE WITH CAP/ASCO GUIDELINES OF 6-48 HOURS FORMALIN FIXATION TIME.

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PATH PROCEDURES

PROCEDURES:

88309, A BLK/16, B BLK/6

FINAL DIAGNOSIS

PART A RIGHT BREAST, SIMPLE MASTECTOMY:

1. INTRALOBULAR AND INFILTRATING LOBULAR CARCINOMA WITH SIGNET-RING CELL DIFFERENTIATION, GRADE II SHOWING NUCLEAR GRADE 2/3 AND LOW MITOTIC INDEX. LCIS OF THE CLASSIC TYPE SHOWS NUCLEAR GRADE 1 COMPRISING APPROXIMATELY 5% OF THE TUMOR.
2. THE INVASIVE COMPONENT HAS A MAXIMUM GROSS DIMENSION OF 5.8 CM WITHIN THE CENTRAL PORTION OF THE BREAST WHICH EXTENDS INTO THE

per TSS, signet ring component <10%.

1 LC 3/2/1
signet ring diff
5.8cm
5% LCIS
classic

Patient: [REDACTED]

(Continued)

Specimen:

Received:

Status:

Req#: [REDACTED]

Collected:

Sp type: SURGICAL P

Subm Dr:

I.D.

ADDENDUM

(Continued)

METASTATIC CARCINOMA IS IDENTIFIED IN ONE OF FOUR ADDITIONAL AXILLARY LYMPH NODES INCLUDING EXTENSION OF TUMOR INTO THE EXTRANODAL SOFT TISSUE (1/4). A SEPARATE AREA HAVING BIOPSY CHANGES IS ALSO PRESENT.

1/4 LN
ENX

Dictated by:

Addendum Signed

(signature on file)

M.D.

Signed _____ Electronically signed by: _____

M.D.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials		
Date Reviewed	10/24/12	

Patient: [REDACTED]

(Continued)

Specimen:

Received:

tus:

Req#: [REDACTED]

Collected:

Sp type: SURGICAL P

Subm Dr:

.D.

FINAL DIAGNOSIS

(Continued)

RETICULAR DERMIS UNDERLYING THE NIPPLE SKIN.

3. THE NIPPLE SKIN, SKIN AND BREAST MARGINS OF RESECTION, AND RANDOM SECTIONS FROM THE REMAINING QUADRANTS ARE FREE OF TUMOR WITH THE CLOSEST LOCATED 4 MM FROM THE NEAREST INKED DEEP MARGIN (A3).
4. LYMPHOVASCULAR INVASION IS IDENTIFIED.
5. FIBROCYSTIC CHANGES INCLUDING SCLEROSING ADENOSIS, MICROCYST FORMATION AND PATCHY DENSE STROMA.
6. CALCIFICATIONS ARE SEEN WITHIN THE TUMOR AS WELL AS BENIGN PORTIONS OF THE BREAST.
7. SCLEROTIC INTRADUCTAL PAPILLOMA.
8. BIOPSY CHANGES ARE IDENTIFIED.
9. SMALL SKIN ULCERATIONS (2).

⊖ marg

Ⓢ LVI

FCC SA

Ca# CA#B9

pap
Bx

PART B RIGHT AXILLA, LYMPH NODE DISSECTION: METASTATIC BREAST CARCINOMA IS IDENTIFIED IN 4 OF 6 AXILLARY LYMPH NODES EXAMINED (4/6). EXTENSION OF TUMOR INTO THE PERINODAL SOFT TISSUE INVOLVES THE LARGEST AREA OF METASTATIC TUMOR WHICH MEASURES 1.1 CM IN GREATEST MICROSCOPIC DIMENSION. A REPRESENTATIVE SECTION OF SKELETAL MUSCLE IS NEGATIVE FOR TUMOR. BIOPSY CHANGES ARE NOT IDENTIFIED GROSSLY OR MICROSCOPICALLY IN THE NODAL DISSECTION.

4/6 + 1/4

ENX

ADDENDUM

Addendum #1

Entered:

THE LATERAL UPPER OUTER QUADRANT IS RE-EXAMINED AND A WING CLIP IS IDENTIFIED IN AN AREA OF FATTY TISSUE, WITH THE TISSUE ANTERIOR HAVING A YELLOW-WHITE INDURATION CONSISTENT WITH A PREVIOUS BIOPSY AREA. THE AREA OF THE CLIP PROPER IS SUBMITTED IN A17 AND 18. THE TISSUE ANTERIOR TO THIS AREA IS SUBMITTED IN A19 AND 20. LATERAL TO THIS, THERE IS A GROSSLY POSITIVE NODAL STRUCTURE 0.6 CM IN DIAMETER. THIS IS SECTIONED AND SUBMITTED IN A21. AN ADDITIONAL POSSIBLE NODE IS SUBMITTED IN A22 AND 23. AN ADDITIONAL POSSIBLE 0.4 CM NODE IS SUBMITTED IN A24.